



Physiotherapy & Rehabilitation Platform

Medical Report: Chronic Lumbar Disc Herniation with Sciatica

Patient Information

Name: ahmet kalumian

Birth Date: 2025-10-28

Case ID: #22

Clinical Details

Doctor: Dr. Alarkany

Date: 2026-04-28

Description: Patient presents with persistent radiating pain from the lower back down to the right calf. Diagnosed with L4-L5 disc protrusion. Pain is aggravated by prolonged sitting and forward bending.

Physical Measurements

Height: 175 cm

Weight: 70 kg

Blood Group: AB+

Health History

Smoker: Yes

Chronic Diseases: Yes

Activity Level: Moderate

Smart Clinical Analysis

Clinical Explanation:

The patient presents with persistent radiating pain from the lower back to the right calf, attributed to an L4-L5 disc protrusion. The pain exacerbates with prolonged sitting and forward bending, indicating a mechanical component to the discomfort. Given the patient's moderate activity level and chronic disease history, a tailored rehabilitation strategy focusing on pain management, mobility enhancement, and strengthening of the lumbar and core musculature is essential.

Immediate Physiotherapy Recommendations:

Initiate a physiotherapy program that includes gentle range of motion exercises, core stabilization, and flexibility training. Emphasize the importance of maintaining proper posture during activities and incorporate modalities for pain relief, such as heat or electrical stimulation, as needed.

Precautions:

Avoid activities that involve prolonged sitting, forward bending, or heavy lifting. Monitor for any signs of increased pain or neurological symptoms, such as numbness or weakness in the lower extremities.

Estimated Recovery Timeline:

Given the nature of the disc protrusion and the patient's chronic condition, a recovery timeline of 6 to 12 weeks is anticipated, contingent upon adherence to the rehabilitation program and response to treatment.

Risk Assessment:

Potential risks include exacerbation of pain, progression of neurological symptoms, or development of chronic pain syndrome if rehabilitation is not appropriately managed.

Red Flags:

Emergency warning signs include sudden onset of severe pain, loss of bowel or bladder control, significant weakness in the legs, or any new neurological deficits.

Test Group: Multiple Sclerosis

Test: Knee Flexion (Right)

Clinical AI Analysis

Doctor Standard/Target:
No doctor target was set

Clinical Interpretation:
The results show a progressive increase in knee flexion from 95 degrees to 115 degrees over the assessment period, indicating improvement in range of motion.

Progress Evaluation:
The patient has demonstrated a positive trend in knee flexion, moving from 95 degrees to 115 degrees, suggesting a recovery approach is effective. The current measurement indicates a significant improvement in mobility.

Physiotherapy Focus:
Continue to emphasize knee flexion exercises, ensuring that they are performed within a pain-free range. Incorporate strengthening exercises for the quadriceps and hamstrings to support knee stability.

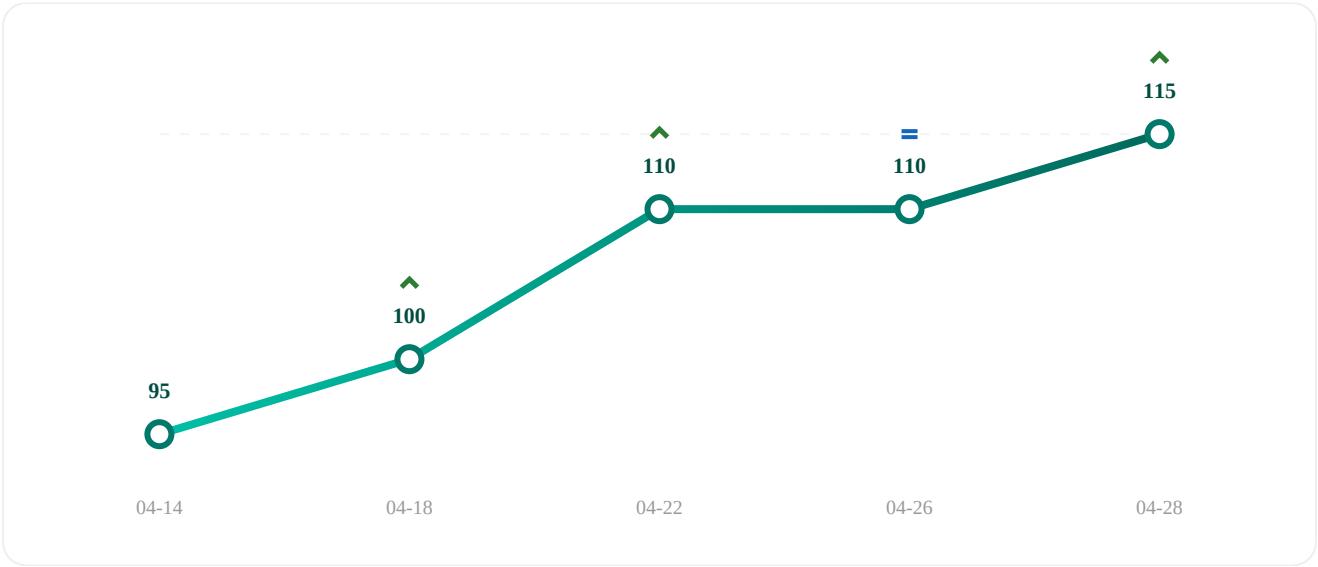
HIGHEST
115.0

LOWEST
95.0

AVERAGE
106.0

OVERALL PROGRESS

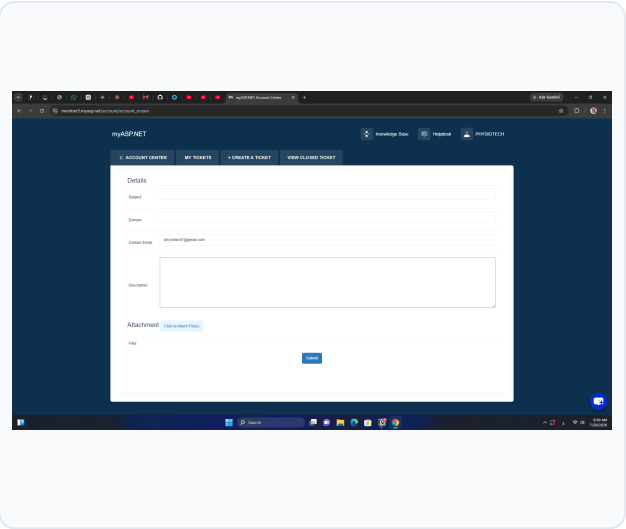
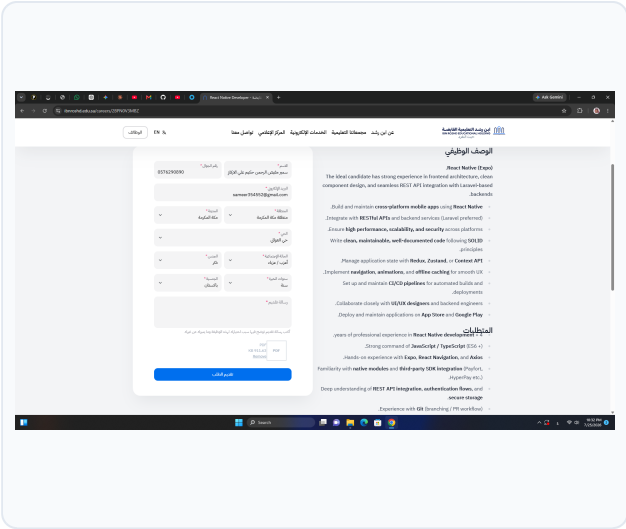
Increased by 21.1% ↑ Since first test (2026-04-14)



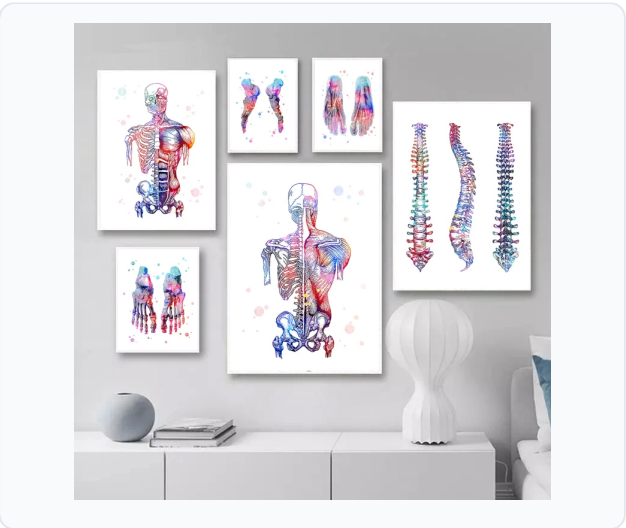
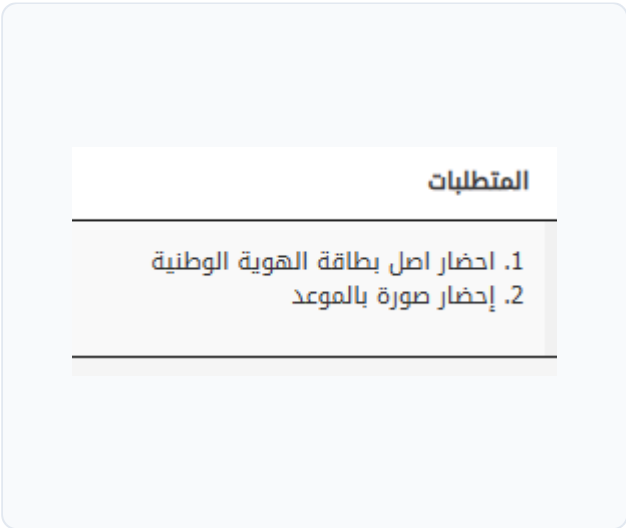
Date	Result	Standard
2026-04-14	95	—
2026-04-18	100	—
2026-04-22	110	—
2026-04-26	110	—
2026-04-28	115	—

Visual comparison: Knee Flexion (Right)

BEFORE



AFTER



Important Notice

This report is AI-assisted and intended for informational support only. It is not a medical diagnosis and does not replace professional clinical judgment. Please consult your doctor or therapist for any medical decisions.

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